MOOD-02 | Therapist only - all age groups

Understanding what limits my day

1 | Purpose, fit and boundaries

Purpose of this unit

Explore one situation in which activity decreased, distinguish practical limitations from a pattern worth exploring, and identify possible support. This builds on MOOD-01 without assuming that every reduction is avoidance.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Rest, recovery and protecting ourselves are not failure. Do not suggest overcoming pain, exhaustion or an unsafe environment through effort. A significant change in functioning or condition needs appropriate assessment, not another form to complete.

Reusing existing material

MOOD-01 records a moment; this sheet explores conditions that limit participation. ASM remains the broader mapping tool. If activity planning is chosen, use the existing CORE-05 instead of duplicating an action plan.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

In what situation did I reduce activity or participation?

2 | Question for discussion

What was actually possible, and what limitations were present?

3 | Question for discussion

What did doing less make possible immediately, and what was its later effect?

4 | Question for discussion

What change in conditions or support is worth exploring?

5 | Question for discussion

What needs exploring before choosing another action?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

Explore with the child what was possible in their environment. Do not make them responsible for transport, money or adults' decisions.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Consider workload, belonging and choices about privacy; do not label time alone as avoidance without exploring it.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Recognize commitments, resources and physical limitations. Rest is one of the available options.

OA | Older adults

Explore accessibility, support and preference without assuming dependence or reduced ability because of age.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

Decide together whether practice between sessions is appropriate at all. Define one opportunity, boundaries, and desired support. Practice may take place only in the session. There is no required daily sequence or requirement to share the sheet with others.

At the next session

Return to the observed barrier and what is needed to understand it. If health assessment or environmental change is needed, do not replace it with an activity goal. Further practice may not be necessary.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

www.nice.org.uk/guidance/ng222/chapter/Recommendations
www.nice.org.uk/guidance/ng134/chapter/Recommendations

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